

Title

Systemic Outcomes and Health Service Utilization in Patients With Atopic Dermatitis Managed Through Coordinated Digital Care

Abstract

Beyond skin-specific outcomes, understanding broader clinical benefits and healthcare system impact informs implementation decisions. Among our 98 participants, secondary analyses examined unplanned healthcare encounters, medication adjustments, and patient activation. The integrated-care group experienced fewer unscheduled clinical visits (median 1.2 vs. 3.1 visits, $p=0.008$) and telephone contacts with clinical staff (median 0.8 vs. 2.4 contacts, $p<0.001$) compared with standard-care participants. Medication optimization, assessed by documented treatment intensification or de-intensification, occurred more frequently in the coordinated-care arm (78% vs. 43%, $p<0.001$). Patient-reported disease knowledge and self-management confidence increased significantly in the intervention group ($p<0.001$). Subgroup analysis suggested larger benefits in participants with moderate-to-severe baseline disease. Cost implications, including estimated reduction in acute-care utilization, favored the coordinated approach despite additional platform costs.